

AHN Allegheny General Hospital

320 East North Ave, Pittsburgh, PA 15212

DISCHARGE SUMMARY Document #: DIS-005

Patient & Admission

Name	Evan Ashley
Date of Birth	1975-07-14
Phone	878-555-7527
Address	148 Eric Track Beaver, PA, 15009
MRN	MRN-543039
Admit Date	2026-04-22
Discharge Date	2026-04-26
Attending	Andrew Reynolds, MD
Attending NPI	3884969650

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	HSB784 801845

Primary Discharge Diagnosis

Type II NSTEMI, demand-related

ICD-10: I21.09, I25.10

Hospital Course

The patient presented to the emergency department with acute onset substernal chest pressure and dyspnea. Initial electrocardiography demonstrated ST-segment depression in the anterior leads with reciprocal changes. Serial troponin I levels were elevated and rising, peaking at 3.2 ng/mL, consistent with acute myocardial infarction. The patient was initiated on dual antiplatelet therapy with aspirin and ticagrelor, along with unfractionated heparin. Coronary angiography performed on hospital day one revealed a 70% stenosis of the left anterior descending artery with evidence of demand-related ischemia; no acute thrombus was identified. Given the clinical presentation and angiographic findings consistent with Type II NSTEMI, percutaneous coronary intervention was deferred in favor of optimal medical management. Transthoracic echocardiography showed a left ventricular ejection fraction of 48% with mild hypokinesis of the anterior wall, and BNP was mildly elevated at 285 pg/mL.

During the hospital course, the patient remained hemodynamically stable on a beta-blocker and ACE inhibitor regimen. Troponin levels trended downward appropriately, and chest pain resolved with medical therapy. The patient was transitioned to oral medications including metoprolol, lisinopril, atorvastatin, and ticagrelor. Repeat echocardiography on hospital day three demonstrated improvement in wall motion with ejection fraction estimated at 52%. The patient tolerated ambulation without recurrent symptoms and demonstrated good

understanding of her medication regimen and lifestyle modifications.

The patient is being discharged on hospital day four in stable condition with close outpatient cardiology follow-up scheduled within two weeks. She has been counseled on medication adherence, dietary sodium restriction, cardiac rehabilitation enrollment, and activity progression. Repeat transthoracic echocardiography is planned in six weeks to assess for further recovery of left ventricular function.

Procedures Performed

Coronary angiography	CPT 93458
PCI to LAD with DES	CPT 92928

Medication Changes

Lisinopril 10 mg daily	STARTED
Atorvastatin 80 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	9 days
Urgency Tier	HIGH
Clinical Flags	post-acute-MI, recent-revascularization, post-PCI dual antiplatelet therapy

Dictated and electronically signed by:
Andrew Reynolds, MD
NPI: 3884969650
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